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Reimagining Global Health

An Introduction

**Paul Farmer
Jim Yong Kim
Arthur Kleinman
Matthew Basilico**



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concluding chapter of *Reimagining Global Health*. Those who have written it have studied and worked together for many years and hope our shared experience will be useful to readers, just as we hope to continue learning from others seeking to pursue the elusive goal of health for all.

Unpacking Global Health

Theory and Critique

BRIDGET HANNA, ARTHUR KLEINMAN

This chapter introduces a “toolkit” of social theories relevant to global health work. We believe that social theory can help students and practitioners understand and interpret the nature, effects, and limitations of medical and public health interventions. As examples in this book illustrate, well-intentioned global health and development projects can have unintended—and at times undesirable—consequences. Careful evaluation of the conditions that enable such consequences can help practitioners design better programs and cultivate a habit of critical self-reflection, which would surely be an asset to global health scholarship and delivery.

Most global health practitioners are focused primarily on *action*: providing services and seeking to improve the health of individuals and populations. Like the many leaders of public health and sanitation initiatives who preceded them, practitioners of global health have rarely had much exposure to, or patience for, the application of social theory to the problems they face. Evaluation of global health work usually focuses on measuring program effectiveness. Social theory has often been relegated to the domain of post-hoc analysis by scholars writing in academic journals, sometimes years or decades after their insights might have been used to improve the delivery of care.

The divide between theory and practice has many roots. Some scholars point to the troubled legacy of political Marxism, in which various interpretations of Karl Marx’s writings were used as the basis for radical, and sometimes violent, reconstruction of the social order. In addi-

tion, the involvement of social scientists, particularly anthropologists, in enabling and justifying the violence of colonialism, and the scientific racism that provided its ideological cover (traced in chapter 3), has led to serious reflection regarding the role of social science in global health and humanitarian work. This history has provided ample cause for the inward turns of deconstructionism and self-criticism that have preoccupied much of anthropology for the past thirty years.

This period also coincided, however, with the rise of medical anthropology as a discipline and the emergence of a new perspective: that of the physician-anthropologist. The physician-anthropologist authors of this volume are examples of those who have used tools from medical anthropology to hone a vision of health equity and social justice. For example, the nongovernmental organization Partners In Health drew on social theory to critique and improve its approach to health care delivery, as chapter 6 explores. It is in this new but vibrant tradition of medical anthropology that we ground our approach to social theory for global health. Effective global health leaders must consider problems from multiple perspectives. They must measure the effects of interventions and explain the meanings of those effects to diverse actors, in diverse places, and at different moments.

Max Weber, an early twentieth-century German sociologist and one of the architects of modern social science, defined sociology as “the science whose object it is to *interpret the meaning of social action* and thereby give a causal explanation of *the way in which this action proceeds and the effect which it produces*.”¹ Weber saw sociology as a science in the sense that it could identify certain causal relationships between social forms. But, as an anti-positivist, he believed that these relationships were not as “ahistorical, invariant, or generalizable”² as those studied by natural scientists. Rather, Weber sought to interpret the meanings of cultural norms, symbols, and values that connect people to structures such as bureaucratic institutions of the state.

Today, anthropology and other modes of social analysis still seek to “interpret the meaning of social action.” The social theories outlined in this chapter can be used to help explain why certain global health initiatives succeed while others fail. These theories elucidate the social determinants of health: the nature and causes of poverty and inequity. The specific social theories discussed here are by no means the only pertinent ones, but they are particularly relevant to navigating the complexities of global health delivery.

BIOSOCIAL ANALYSIS AND THE SOCIOLOGY OF KNOWLEDGE

Most medical research focuses exclusively on the biologic causes of disease. A biosocial approach posits that such biologic and clinical processes are inflected by society, political economy, history, and culture and are thus best understood as interactions of biological and social processes. Biosocial analysis of global health challenges reaches across disciplines and breaks down the boundaries that separate them; for example, understanding questions of resource optimization, which are usually reserved for economists, also requires insights from anthropologists and health practitioners. One central illustration of the biosocial nature of disease is the correlation between disease risk and poverty. We will revisit this relationship often in this volume under various names (such as structural violence and social suffering) and in the context of different diseases (such as malaria, AIDS, and multidrug-resistant tuberculosis).

Other concrete examples include the link between psychological and economic depression and the rise in rates of disability associated with increased unemployment. Likewise, economic status, education level, cultural traditions, and access to infrastructure all influence dietary habits, a crucial determinant of heart disease and obesity. The global epidemic of type 2 diabetes mellitus is attributable in part to greater sugar intake. To add complexity to the issue of culture and diet, one must also consider mental health. For example, Anne Becker and colleagues conducted dietary studies in Fiji and found that as tourism grew and infrastructure was developed, the diets of the native people shifted toward Western eating habits. Rates of anorexia and bulimia rose as television watching became popular and displaced other cultural events and large family meals.³ None of these phenomena can be examined properly in the absence of biosocial analysis.

A biosocial approach demands the reconciliation and occasional disruption of multiple frames of knowledge. A medical student will learn, for instance, that the cause of cerebral malaria is the protozoa *Plasmodium falciparum* and that the treatments of choice include quinine or artemisinin-based combination therapy, whereas an epidemiologist or a public health planner might view the cause as undrained breeding grounds for the mosquito vector and recommend draining standing water, providing mosquito nets, and spraying DDT. All the while, residents of an endemic area and ethnographers who speak with

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them might assert that the cause of malaria is the unequal distribution of land under the local system of tenancy and might protest that DDT's environmental effects make its use unacceptable.

How should global health practitioners navigate these multiple ontological claims? Differential values assigned to diverse categories of evidence—in this case, whether the problem is deemed biological, environmental, or economic—will shape the proposed solution and its ultimate effect. This is particularly clear in the case of malaria. Although the biological view of malaria causality largely won out over the geographic and sociological views in the 1970s, some historical analyses of malaria burden suggest that land development and distribution are as important as technological interventions in eliminating malaria.⁴ To help clarify these multiple explanatory frameworks, we introduce biosocial perspectives and the sociology of knowledge.

In 1966, sociologists Peter Berger and Thomas Luckmann published *The Social Construction of Reality*, which defines the sociology of knowledge as “whatever passes for knowledge in a society, regardless of the ultimate validity or invalidity (by whatever criteria) of such ‘knowledge.’”⁵ Berger and Luckmann begin by explaining how people form shared mental conceptions about the world: when any group of people—whether they are sailors marooned on a desert island, first-year medical students, or commodities traders on Wall Street—find themselves together, they construct norms to govern their relations. As jokes, habits, and practices are passed on to subsequent generations, they become freighted with meaning and assume the status of immanent rules. If these individuals have children or initiate others into their community, the newcomers will over time experience these historicized habits as natural rules. Berger and Luckmann call this process *institutionalization*, in which “reciprocal typification of habitualized action by types of actors” leads to the eventual objectification of that habitualized action as an institution.⁶ Assumptions and accidents become historicized into truths, and knowledge is created.

“To understand the state of the socially constructed universe at any given time, or its change over time,” Berger and Luckmann write, “one must understand the social organization that permits the definers to do their defining. Put a little crudely, it is essential to keep pushing questions about the historically available conceptualizations of reality from the abstract ‘What?’ to the socially concrete ‘Says who?’”⁷ It is through this process that people’s knowledge and beliefs about the world—all human knowledge, including science, “regardless of its ultimate valid-

ity or invalidity”—become legitimized in society, and the world can be said to be “socially constructed.”

The mechanisms of legitimation are useful to keep in mind in any field; for global health analysis, legitimation helps explain how practices become institutionalized. When transformed into policies backed by organizations with claims of authority, legitimized knowledge comes to exert social control over individuals. People feel pressure to obey rules and conventions that have become dissociated from human agents and are instead imbued with coercive power because they have been legitimated and institutionalized. For example, individuals may choose to exercise and follow a “healthy” diet because public health norms recommend these behaviors; doctors may be unable to give patients an already-proven vaccine because it hasn’t yet received World Health Organization pre-qualification (a stamp of approval for global health interventions). “Man,” in other words, “is capable of creating a world which he experiences as something other than a human product.”⁸ Social constructions become naturalized over time, as if they were invariant parts of the nature of things.

Even demonstrable scientific principles, including those established beyond doubt, are socially constructed in the sense that they remain historical products of specific questions and experiments—and of human minds, which, in an alternate history, might have asked other questions and conducted different experiments. The *Diagnostic and Statistical Manual of Mental Disorders (DSM)* is a good example of the social construction of knowledge in medicine. Given that it is an authoritative text designed to guide practice and treatment norms for psychiatry in the United States, and to some degree worldwide, one might assume that it would be immune to cultural variance. But until 1973, the *DSM* maintained that homosexuality was a psychiatric disease.⁹ In this case, social biases shaped medical diagnosis. Similarly, the *DSM* has redefined and downgraded the amount of time that is considered “normal” for someone to grieve after the death of a spouse or child. In the past, a person who still felt intense symptoms of grief a year after such a personal tragedy would have been considered depressed. But in recent decades, stigma associated with depression has decreased, and the use of psychopharmaceutical drugs has become more widely accepted. According to today’s *DSM*, a person is considered clinically depressed if he or she still feels grief after two weeks. This arbitrary process of *medicalization*, whereby subjective experiences are redefined as disease—such as remaking war trauma into post-traumatic stress

disorder, or severe cases of premenstrual syndrome into premenstrual dysphoric disorder—illustrates the social construction of knowledge and the institutionalization of medical norms.¹⁰

As we grapple with the social construction of medical knowledge, a technical distinction drawn from medical anthropology might be useful. While in general parlance the terms “illness,” “disease,” and “sickness” are used interchangeably, medical anthropologists posit distinctions between them. Illness can be understood as the subjective experience of symptoms by laypersons and their communities,¹¹ disease as the reinterpretation of these symptoms as objective categories by medical practitioners, and sickness as pathology at the population level.¹² Awareness of how both knowledge and policy are socially constructed helps students and practitioners take a critically self-reflective approach to global health delivery.¹³

THE UNANTICIPATED CONSEQUENCES OF PURPOSIVE ACTION

Awareness of the social construction of knowledge, however, does not illuminate how even well-intentioned initiatives can unwittingly cost lives and resources. Robert Merton’s theory about the *unanticipated consequences of purposive social action* offers insight into this phenomenon. Purposive action, according to Merton, involves motives and, consequently, a choice among alternatives; it must also have a goal and a process.¹⁴ Nevertheless, such an action may not achieve the desired aim and may in fact result in unanticipated, and sometimes undesirable or perverse, outcomes. Unintended consequences of purposive action vary, as do their causes. One potential cause is knowledge asymmetries: for example, a doctor might misunderstand the language or cultural traditions of a patient and misdiagnose or mistreat the patient accordingly. Even with all relevant information, one can always make an error or take an action that subverts one’s ultimate goal.

Merton also identifies “rigidity of habit” on the part of individuals or institutions and “the imperious immediacy of interest” as potential causes of unanticipated consequences.¹⁵ For example, the United Nations responded to the “imperious immediacy” of the plight of refugees fleeing the 1994 Rwandan genocide by setting up refugee camps in neighboring Democratic Republic of the Congo (shown in figure 2.1). Those camps, however, became a base of operations for the perpetrators of the genocide—a devastating and unintended consequence that helped launch frequent bouts of violence that continue to this day.¹⁶



FIGURE 2.1. This Rwandan mother and her two children, refugees from the Rwandan genocide, stand in front of their sheeting shelter overlooking the Kibumba refugee camp, twenty miles northeast of Goma, Democratic Republic of the Congo, on February 13, 1996. Organized by international humanitarian organizations, such refugee camps, located along the border between Rwanda and the Democratic Republic of the Congo, unintentionally became bases of operation for Hutu militias that continued to systematically slaughter Tutsis in the camps and surrounding regions. Courtesy Associated Press/Jean-Marc Bouju.

Institutional values can also prevent us from anticipating possible outcomes. In the example of the Péligre Dam in Haiti, discussed in chapter 6, the values of international development institutions blinded the builders of the dam to the fact that the project would displace whole communities and trigger poverty and homelessness. Most large infrastructure projects, from dams to highways to power plants, have consequences both intended and unintended.

Finally, Merton explains that in some cases merely announcing one’s intentions can alter the circumstances surrounding an action; even the best plans are laid on shifting ground. For example, in places where decent medical care is scarce, simply announcing plans to build a new hospital, or to upgrade an old one, can cause a surge of patient visits to the site before the new facility is ready. On-site clinical staff members, if there are any, must suddenly care for hundreds of additional patients

each day, likely in outdated facilities that had previously handled only a few dozen patients each day. This is just one example. Realized and potential unintended consequences are too numerous to count, and their impact looms large in the history of global health.

THE RATIONALIZATION OF THE WORLD

Berger and Luckmann demonstrate how social institutions and legitimized knowledge shape the agency of individuals; Merton asks why purposive social actions often fail to achieve their intended result. Today, it is rare for individuals to be the sole, or even the primary, actors in an intervention. Institutions and organizations—governments, nongovernmental organizations (NGOs), corporations, and multinational organizations such as the World Bank and various United Nations agencies, including the World Health Organization—are more frequently the architects of global health practice and policy.

Embedded in the actions of institutions and some individuals are power and authority (or the lack thereof, as is the case for many of the patients this volume describes). Max Weber (pictured in figure 2.2) delineates three modes of authority.¹⁷ *Traditional authority* such as patriarchal, patrimonial, or feudal power derives from history, custom, or (in Berger and Luckmann's words) institutionalization. It is the power passed on from generation to generation by monarchs, barons, village headmen, and tribal chiefs.

Charismatic authority is generated by extraordinary leaders capable of mobilizing large numbers of people around an idea or goal. It can be associated with religious leaders (such as Buddha, Jesus, Krishna, Muhammad, or Moses), political leaders (such as Nelson Mandela or, on the other end of the spectrum, Adolf Hitler), and leaders of moral movements (such as Mahatma Gandhi, Mother Teresa, or the Rev. Martin Luther King Jr.). Although charismatic and traditional authority are often contrasted, they can also overlap: the religious traditions inspired by charismatic leaders take on traditional authority based on custom after their founders die; many moral movements are also political in nature. Charismatic authority, though difficult to quantify, can be an important element of a successful global health endeavor. Mobilizing a team and attracting internal and external support, both critical for fledgling projects, may depend on the irreplaceable, unquantifiable attraction and effectiveness of a particular activist or leader. As Weber writes, such leadership rests “on devotion to the exceptional sanctity,



FIGURE 2.2. Max Weber, an influential social theorist who shaped our understanding of bureaucratic institutions, charismatic authority, the process of rationalization, and other social phenomena.

heroism or exemplary character of an individual person, and of the normative patterns or order revealed or ordained by [that person].”¹⁸

Most central to our work here is Weber's third type of power, which he called *rational-legal authority*, a category inclusive of modern law, the state, and organized institutions in which the authority of the ruler derives from laws and rules. This kind of modern authority functions in the context of what Weber terms *bureaucracy* (refer to table 2.1). Bureaucratic power, he argues, is fundamentally different from other kinds of power, deriving not from tradition or charisma, both of which are often vested in individuals, but from institutions. Bureaucrats—members of the bureaucracy—are parts of organizations that can replace them and will likely outlive them. As Weber predicted, the replaceable nature of bureaucrats resulted in an increase in programs granting certificates and degrees for specific, and increasingly specialized, jobs. Bureaucracies have a hierarchical structure of subordination: vocations and responsibilities are clearly defined and correspondingly compensated; and the individual, Weber writes, is a “single cog in an ever-moving mechanism which prescribes to him an essentially fixed route of march.”¹⁹

Weber predicted that institutions would become the most powerful social structures in society, greater than family or community,

TABLE 2.1 MAX WEBER: MODES OF AUTHORITY

Type of Authority	Derives Its Power From
Traditional	History, custom, and habitualization over generations; accepted because “it is how things have always been”
Charismatic	Extraordinary leaders who can mobilize people around an idea or goal
Rational-legal	Consistent application of a set of rules and laws, whose execution depends on a system of bureaucracy, characterized by: • Fixed and official jurisdictional areas governed by formal rules • A hierarchy of authority composed of a system of supervision and subordination • Maintenance of files and records • Official activity distinct from private life (separation of office and officeholder) • Technically qualified personnel operating at full capacity • Specialization of tasks and division of labor

because they could both generalize and quantify. This development, he argued, would result in the technologization and bureaucratization—the *rationalization*—of everyday life. Describing “the disenchantment of the world,” Weber explains how rationalization transforms the mystical and the mysterious into laws, rules, and regulations (in a manner similar to Berger and Luckmann’s process of institutionalization). Protocols, technical jargon, neologisms, simplifications, standardizations, and scientific methods are all part of the rationalization that Weber believed would come to dominate the modern world and would increasingly legitimize bureaucratic power over both traditional and charismatic power. Think, for instance, of how common sense and generalized ideas of danger are now reified as specific ideas of defined risk that entail risk appraisal, categorization, management, forecasting, insurance, and prevention.

Rationalization, Weber recognized, has positive potential, yet can be very dangerous. It is unparalleled in its efficiency as a tool for administering large and complex systems because it is more generalizable and quantifiable and less ad hoc than other types of power. Although there is the ever-present possibility of corruption within bureaucracies, they are usually much more egalitarian systems than traditional or charisma-based ones. While a hereditary king or a charismatic sect leader might require certain familial connections or religious beliefs in order to allow an individual access to traveling papers or educa-

tional opportunities, bureaucracies tend to require legal—rather than ideological—authorization.²⁰

Weber also presaged certain dystopian implications of the rationalization of the modern world. Bureaucracies at times function like an “iron cage,” in which rules trump common sense, creative innovation, and human decency. Individuals working in bureaucracies have little incentive to change or improve the rules because their jobs are contingent on efficiently executing—not questioning—the specific tasks assigned to them. Once created, bureaucracies are thus difficult to reform or destroy: a large number of individuals have a stake in their preservation and constancy. Amid these and other observations about the modern world, Weber imagines the rising tide of rationality leading into a “polar night of icy darkness.”²¹

Understanding the benefits and the dangers of bureaucratic rationality can sharpen scholarship and practice in global health. When we look at the institutions that govern international efforts in global health—such as the World Health Organization (WHO), the United Nations Children’s Fund (UNICEF), and the U.S. Agency for International Development (USAID)—it is easy to spot both the tactical advantages that a bureaucratic structure gives them and instances in which rule-bound behavior leads to everything from improved outcomes (the intended consequence) to inefficiencies and even, at times, grave missteps. An example of the latter is the WHO’s use of cost-effectiveness analysis to formulate policy relevant to the AIDS pandemic. Despite the existence of effective therapeutics since the mid-1990s, as we will see in chapter 5, AIDS was declared too expensive to treat among poor people until the early 2000s. Similar judgments have been made by global health policymakers about treating other chronic diseases such as multidrug-resistant tuberculosis (MDRTB), diabetes, or depression in low-income settings. Chapter 8 describes in depth how a narrow focus on cost of treatment per life-year saved has hamstrung the global response to MDRTB. Weber’s notion of bureaucracy informs our discussion of global health policy and practice throughout this book, as does the brief consideration of Michel Foucault’s work on disciplinary power in the next section.

DISCIPLINE AND BIOPOWER

History is written by the victor, or so the saying goes. This aphorism, and many like it, speak to the relationship between knowledge and

power. Through the concept of *biopower*, French philosopher and historian Michel Foucault seeks to explain how biologic and medical data are used by the institutions of the modern world to define, count, divide, and—in a word—*discipline* populations. Biopower is another important addition to our theoretical toolkit for global health.

Foucault analyzes, among other things, the history of seventeenth- and eighteenth-century cultural and political institutions like the insane asylum, the prison system, and the clinic. He traces how these institutions constructed norms about what is considered sane and insane, licit and illicit, healthy and sick, which later generations inherited and, over time, accepted as natural. For example, the criteria for deciding whether someone is insane and should be sent to a hospital or criminal and should be sent to prison evolved from a set of institutional practices into norms, some of which became enshrined in law.

Similarly, the idea of prisons as places for reform rather than punishment evolved over time. Foucault's book *Discipline and Punish* begins with an unforgettable torture scene from eighteenth-century France. A prisoner named Damiens is taken to a scaffold, where it is ordered that "the flesh will be torn from his breasts, arms, thighs and calves with red-hot pincers, his right hand, holding the knife with which he committed the said parricide, burnt with sulphur, and, on those places where the flesh will be torn away, poured molten lead, boiling oil, burning resin, wax and sulphur melted together and then his body drawn and quartered by four horses and his limbs and body consumed by fire, reduced to ashes and his ashes thrown to the winds."²²

According to Foucault, this gruesome punishment served a chiefly symbolic function. The king, the body of the state, had been attacked, and therefore the perpetrator's body would be attacked in return, beginning with the hand used to commit the treasonous act. Foucault terms this public display of coercive force *sovereign power*, "the right to *take* life or *let* live."²³ Weber might have classified this modality of power as traditional authority because it was reinforced by tradition and sanctified by religion.

Foucault compares this form of punishment to the prison systems that began to appear a few decades later, in which a different form of disciplinary power emerged as a tool of correction. Observing, reforming, converting, and categorizing became the mechanisms of discipline within prisons. Other institutions such as hospitals, asylums, and the bureaucracies of the state also began employing these techniques over

the course of the late eighteenth and nineteenth centuries. During outbreaks of plague, for example, public health authorities developed an infrastructure of surveillance and quarantine for observing and collecting information about populations and individual bodies. After epidemics of plague subsided, states continued to use this infrastructure to keep track of, and exert coercive force over, their subjects. In the modern world, Foucault posits, disciplinary power is the principal means by which governments and other coercive institutions control populations.²⁴

In 1785, Jeremy Bentham designed a theoretical prison called the panopticon, which illustrates the mechanisms of disciplinary power. A hexagonal prison with windowed cells around the perimeter and a dark guard tower at the center, the panopticon was designed so that inmates were always visible from the tower without knowing whether they were being watched. The possibility of being watched would lead inmates to correct their behavior automatically. The coercive force generated by the panopticon—constant self-surveillance and correction among those inside—is a form of disciplinary power. Foucault argues that the institutions of modern society exert a similar power; they discipline individuals instead of coercing them directly.

Foucault's concept of discipline helps us to understand biopower. The sovereign whom Damiens tried to kill had traditional authority, but the king's power was limited in important ways. He had the right of seizure and the right over life—that is, he could claim a certain amount of his subjects' labor or crops as tax, and he could kill a person for being a traitor or force men to die at war in his army. But the king did not know, or seek to know, what went on within the walls of people's homes, beds, prison cells, or bodies.

In the eighteenth century, however, along with the prison reformers came a rationalist revolution focused on quantifying and documenting many aspects of life—from anatomic dissection to classification to collecting census data to developing statistical analysis—often driven by the goal of consolidating power over life. Centralized bureaucracies consolidated power in France by counting and controlling the health and social welfare of populations, moving from sovereign power to what Foucault called *governmentality*. These activities had many names and purposes, as they do today. Some employers document their employees' eating habits, for instance, in order to promote healthy living and make the workers more efficient; states require that children receive vaccinations before they attend school to improve population

health and student performance. These acts carry state power and governance—the reach of governmentality—into ordinary lives and into the body itself.

Biopower, then, can be seen as the form of governmentality that deals with life. It is embedded in the processes of modern capitalism. Unlike sovereign power, biopower is diffuse and does not operate through specific visible agents. Likewise, biopower may be said to exert control over life as opposed to death: it brings “life and its mechanisms into the realm of explicit calculations and [makes] knowledge-power an agent of transformation of human life.”²⁵

According to Foucault, biopower emanates from two poles: first, the regulation of biologic processes (propagation, health, longevity, mortality) at the level of the population; and, second, technologies for discipline at the level of the individual (such as the panopticon) (see figure 2.3).²⁶ It is an essentially productive power, in that it endeavors to administer, optimize, and multiply knowledge about populations, subjecting them to precise controls and comprehensive regulations (by, for example, measuring the size and distribution of populations or categorizing populations by gender, age, race, occupation, fertility, mortality, and so on). Biopower is at work, therefore, any time quantification of life leads to categorization of life.

Manifold examples from the colonial and contemporary periods demonstrate this type of power. From anthropology, there is the history of using anthropometric measurements of the skull and a racial “science” to construct an invidious evolutionary ladder with Africans at the bottom and Europeans at the top. The colonial construction of stereotyped and institutionalized divisions between ethnic groups is a source of violence and conflict to this day. In Rwanda, Belgian colonists branded the minority Tutsis as a military and royal caste, in opposition to the Hutus, whom they categorized as peasant farmers. The distinctions the Belgians reinforced helped fuel ethnic conflict; decades later, the categories Hutu and Tutsi were the fault lines of the 1994 genocide.²⁷ Similarly, the British codified caste differences among the Indian population, a legacy that contributes to continued caste violence and inequality in India today.²⁸ There are few straight lines in history, and we must be wary of claims of causality in complex social fields like those in which ethnic violence occurs. Nonetheless, it would be naïve to disregard the long-term impacts of colonial policies, stereotypes, and hierarchies on postcolonial societies and politics today.

Biopower is not necessarily destructive, however. The two most

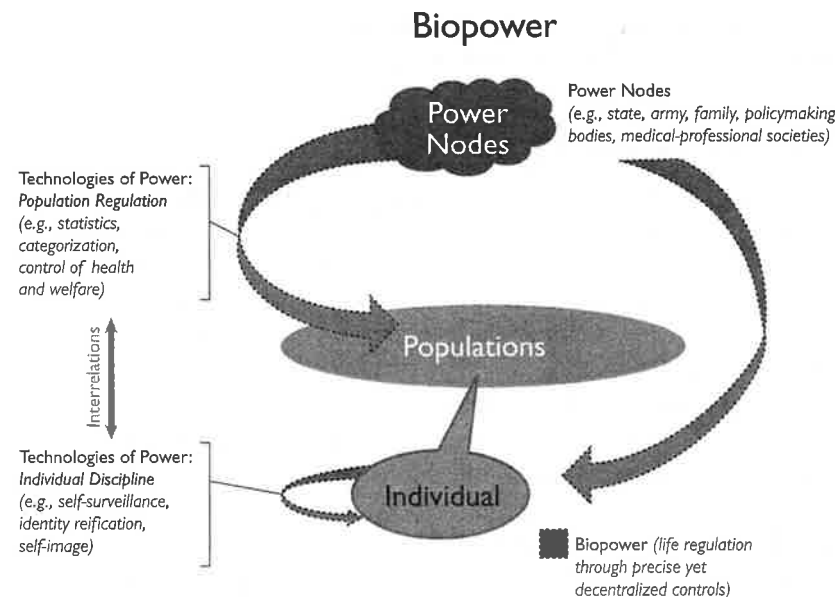


FIGURE 2.3. Biopower, a concept developed by Michel Foucault, helps us understand how the quantification of individuals’ biology contributes to the discipline of the body and the regulation of modern life.

notorious industrial disasters of the past century, the 1984 Bhopal gas leak in India and the 1986 Chernobyl nuclear meltdown in Ukraine, illustrate how categorizing disaster-related illnesses can create new subjectivities and state policies. In Bhopal, state bureaucracies not only based disability compensation and medical care on illness categories that excluded the long-term effects of gas exposure but also demanded documents that the poor could neither access nor produce.²⁹ In contrast, while according to some estimates only two thousand people were affected by the Chernobyl disaster, fully one-third of Ukraine’s population secured enrollment in the Chernobyl compensation scheme. The collapse of other state disability systems in the wake of Ukraine’s withdrawal from the Soviet Union had created a deficit in services, which was partially remedied by the system created to care for Chernobyl survivors. Adriana Petryna calls this phenomenon “biological citizenship,” a “massive demand for but selective access to a form of social welfare based on medical, scientific, and legal criteria that both acknowledge biological injury and compensate for it.”³⁰ Hence, an important component of our toolkit is the critical examination of processes through

which biopower operates in global health interventions and shapes our understanding of these interventions.

SOCIAL SUFFERING AND STRUCTURAL VIOLENCE

The study of global health consists of more than examining specific programs and interventions. At stake are the lives and livelihoods of millions of people and families. Although biopower may help identify the limits and conditions of knowledge, it may not help us understand suffering, the question of who suffers most, or why one person suffers and another does not.

Arthur Kleinman, Veena Das, and Margaret Lock developed the term *social suffering* to account for the forms of social violence that constitute inequity. “Social suffering,” they write, “results from *what political, economic, and institutional power does to people* and, reciprocally, from *how these forms of power themselves influence responses to social problems*.”³¹ In other words, institutions and their agents can perpetrate violence in the name of health and welfare. Social forces—including economics, politics, social institutions, social relationships, and culture—can cause pain and suffering to individuals.

Being born into poverty, facing discrimination because of the color of one’s skin, or living in an abusive home are all dimensions of social suffering. The term also encompasses the interpersonal experience of suffering, the experience of chronic illness, and the ways in which society and its institutions unintentionally exacerbate social and health problems. The concept of social suffering addresses the intersection of medical and social problems—for example, the need for coordination of social and health policies in response to the clustering of inner-city violence, substance abuse, depression, and suicide.

We close with a theory that addresses the roots of global health inequities. Paul Farmer’s observations of the links between poverty and ill health in Haiti informed the development of the concept of *structural violence*, which can be thought of as a form of social suffering. “Such suffering,” he writes, “is ‘structured’ by historically given (and often economically driven) processes and forces that conspire—whether through routine, ritual, or, as is more commonly the case, the hard surfaces of life—to constrain agency. For many, including most of my patients and informants, choices both large and small are limited by racism, sexism, political violence, and grinding poverty.”³²

Farmer describes the plight of women living with AIDS in rural

Haiti. Without understanding the forms of structural violence at play in this setting, one might presume that these young women had contracted the disease because—to use language that casts them as free agents—they chose to be promiscuous. This conclusion, however, would be wrong on two scores. First, at the time of Farmer’s research, the main difference between women in rural Haiti who had HIV and those who did not was the occupation of their primary sexual partner. With few opportunities to earn income themselves, women often partnered with drivers or soldiers, men whose mobility and status brought increased income. But these men were also more likely to have multiple girlfriends or sex partners, especially in urban areas, who in turn were more likely to have had contact with sex tourists from the United States and other countries. The rural female partners of these men, therefore, were more at risk of contracting HIV than were those women whose partners were local farmers. Neither group of women could be classified as “promiscuous.” When structural violence is overlooked, agency is often overestimated, constraint underestimated. “Attentiveness to the life stories of women with AIDS,” Farmer observes, “usually reveals that their illness is the latest in a string of tragedies.”³³

Structural violence helps deconstruct why for so many people suffering with disease and disability, an illness such as AIDS is but one additional misfortune piled on previous layers of hardship. At the macro level, the theory underscores the political, economic, and historical forces that pattern and link material deprivation and poor health.

CONCLUSION

This toolkit of social theories is by no means comprehensive, but its primary areas of focus—knowledge, power, institutions, and inequity—are all central to the study and practice of global health. Social theory provides an organizational framework for global health. These theories will not cure tuberculosis, bring a baby safely into the world, or care for the elderly. But illuminating some of the relationships that govern social action can help us design better programs, guide practical solutions to health challenges, and develop habits of critical self-reflection among practitioners.

Moreover, drawing on the biosocial framework that animates this textbook, we share an even more ambitious goal. We seek to show how approaches to global health problems can benefit from social theory,

which helps us understand the dynamic relationships between those problems and the interventions launched to counter them. That is why theory matters. We hope, too, to convince the reader that the assortment of problems that constitutes global health can be conceptually framed in such a way as to develop global health as an interdisciplinary, academic subject.

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Colonial Medicine and Its Legacies

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The groundswell of interest in global health issues over the past few years sometimes causes observers to see the field as "new." There are indeed many new features that characterize global health concerns in the twenty-first century, such as the worldwide visibility of the AIDS pandemic and the reshaping of global biosurveillance in the wake of the SARS and H1N1 epidemics. Many of the therapeutics in the medical arsenal are also new and far better evaluated than their predecessors. But transnational and pandemic diseases are not new, even when the ability to track them—for some, an example of biopower—becomes more sophisticated or involves new diagnostics for previously undescribed (or truly novel) pathogens.

The new, as always, is rooted in the old, and serious biosocial exploration of global health today would be incomplete without plumbing history. Even a cursory evaluation of current global disparities in disease burden and access to biomedical therapeutics makes clear that efforts to improve global health equity must navigate a landscape littered with the wreckage (and the occasional glorious monument) of programs past. In turn, many of these failures—such as the global malaria eradication campaign of the 1950s and 1960s (discussed in this chapter)—may be attributed, in part, to a lack of historical reflection and biosocial analysis. Like the social theories considered in the previous chapter, history can help us understand the intended and unintended consequences of global health interventions.¹